طبع فى 23:22:07 24/10/2026

جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : منيره احمد السيد محمد
المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** النحاسين
رقم البطاقة : 27311121303008 **رقم التليفون :** 01141537756 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 12-11-1973 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: منيره احمد السيد محمد
National ID: 27311121303008

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



منيره
احمد السيد محمد احمد
١ المروه-ش الرشاح-سيد غريب
اول السلام - القاهره

١٩٧٣/١/١٧ ٢٧٣ ١١ ١٢ ١٣ ٠٣٠ ٠٨

IT5544889

مسد (١)
فادلودين + ايبراز
محذرى

٢٧٣١١١٢١٣٠٣٠٠٨ ٢٠٢١/٠٢
ربة منزل

أنثى مسلمة متزوجة
محمد عبدالعزيز السيد على
١٠ البطاقة سارية حتى ٢٠٢٨/٠٢/٠٧

National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

MDT FORM 21/12/2022 GA

Name	مني [illegible]	MRN	٢٠٢٢/٢٦٩٣
Age	49 year	ID	٢٧٢١١١٢١٢٠٠٠١
Menopausal status	post menopausal	Phone	٠١٠٩٤٠٩٨٥٩٣
NRS SCORE		marital status	
BMI		Family history	١٥٠٣٠٢٠٠٢٤٦
Co morbidity	NO	REASON OF MDT	

Clinical Data

Bx: Lt cervical L.N: GII mammary ductal origin Gata 3: +ve.

ER:	PR:	HER 2:	KI67:
7/8	8/8	-ve	10%

Sonomammography :

Mets Workup: C.T 9/2022: Free
- Lt 7th rib benign looking lesion.
PET C.T: 11/22: low grade active LV 2-3 osseous lesions ~~under tet~~

R / L

TNM

Recommendations:
2cig. Lt Breast CA yrAC + 12wk taxol 1/2022 → Lt MRM : T2N3 &R1 PR + HER2 -ve → PORTH → Neoadjt till now in 9/2022: new Lt cervical mass.

MDT OUTCOME & PLAN Oligo-recurrent disease (Cervical LN3) → Metastatectomy done on (13/10/22): - 2ft end resistance.
for: ① Post-op neck U/S ② T. markers ③ Hormonal profile (FSH, E2, ...
to shift to AI. +/- oophorectomy (according to menopausal status).

MEDICAL ONCOLOGY	SURGERY	SIGNATURE RADIOLOGY	PATHOLOGY	Nutrition	psychiatric
د/ [illegible]	د. [illegible]	[illegible]			

٢٠٢٢/١٢/٢١

Name: منيره احمد السيد **MR. :** 966	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 52-year-old individual with a history of post-mastectomy locoregional recurrent breast cancer involving the left cervical region. The disease course includes pulmonary nodules measuring 5 mm identified in 2022, a benign lesion of the left seventh rib, and a left cervical lymph-node excisional biopsy confirming invasive ductal carcinoma grade 2, estrogen-receptor score 7/8, progesterone-receptor score 8/8, HER2-negative, and Ki-67 10 %. Prior treatment comprised left breast cancer surgery in 2020, neoadjuvant chemotherapy with anthracycline and taxanes, left modified radical mastectomy in January 2020 (pathologic stage T2 N3), postoperative radiotherapy, and adjuvant tamoxifen continued until October 2022. Subsequent locoregional recurrence in the left cervical lymph node was managed with letrozole, and computed tomography on 17 June 2026 demonstrated mild posterior-density changes of a right middle-lower lobe pulmonary nodule measuring 15 × 6 mm, as well as stable bilateral pulmonary nodules.

Biopsy of the left cervical lymph node confirmed invasive ductal carcinoma grade 2, ER 7/8, PR 8/8, HER2-negative, Ki-67 10 %. Imaging studies reveal a 5-mm pulmonary nodule noted in 2022, a benign left seventh-rib lesion, and on recent CT a 15 × 6 mm posterior-density nodule in the right middle-lower lobe with stable bilateral pulmonary nodules, without evidence of further metastatic disease.

The MDT recommends initiating therapy with ribociclib (Ibrance) combined with fulvestrant (Faslodex), with consideration of adding goserelin (Zoladex) based on the patient’s menopausal status.

تحريرا في ,

التاريخ :	**23-09-2026**	**الوقت :**	

الطبيب المعالج :

يعتمد ,

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

08/02/2026

MDT FORM

Name:	Monira Ahmed El-Sayed .	MRN:	966
Age:	53 Y	ID:	27311121303008
Menopausal Sta		Phone:*	01098503388
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender			
CO Morbidity:		Reason Of MDT:	locoregional recurrent luminal bc with quiny pulmonary nod with recent pd of pu
Surgical History:* mrm		ECOG Performance Status	○ 0 ● 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX:

ER: PR: HER2: KI67:

TNM

History:
postm locoregional recurrent breast cancer (left cervical) with quiny pulmonary nodules 5 mm from 2022 (left 7th rib benign lesion) left cervical ln excisional biopsy idc 2 er7/8 pr8/8 her2-ve ki 10% history left breast cancer 2020 rec nact ac taxanes left mrm 1/2020 t2 n3 mo rec port ocivadex till 10/2022 locoregional recurence ileft cervical ln rec femara cts pul nod 5 mm bs benign left 7 th rib cts 17/6/2026 mild pd of rt middle lower lobe pulmonary nodule m 15x6 mm sd of rest of bilat pul nodules for mdt

	Right Side				Left Side			
	Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Sonomammography:

Biomarker/Molecular

Clinical Staging:

Mets Workup:

Pathological Staging:

Recommendations:

MDT Outcome & Plan
for ibrance faslodex+/- zoladex according to menopausal status

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
Dr Manan Ezzat			

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient's Name:	**Moneera Ahmed El-Sayed Mohammed**
Date:	**2026-06-17**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA underwent left MRM. Compared to the previous study dated 21-8-2025, the current study showed:

I- CHEST:

- Mild progressive course since last study regarding the size of the previously seen right middle lobe measuring 15 x 6 mm.
- Rather stationary course regarding the size and number of the rest of the previously seen bilateral scattered pulmonary nodules.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- No appreciable time interval changes regarding the previously seen left 7th rib exophytic lesion with internal calcifications compressing and scalloping the left 6th rib.

Opinion:

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Moneera Ahmed El-Sayed Mohammed**
Date:	**2026-06-17**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- **Mild progressive course regarding the size of the previously seen right middle lung lobe.**
- **Stationary course regarding the rest of the study.**

Dr.sherif Wagdy

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Monira Ahmed Al Sayed Mohamed	Date : 21/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 51 Year(s)
Medical ID: 3693/2022	Pat. ID: 19987

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS: -

Findings:

Reported case of breast cancer compared to last study dated 25/3/2024

I Chest:-

- Status post left MRM showing no sizable operative bed recurrent lesions.
- No pathologically enlarged axillary lymph nodes.
- **Mild progression** of bilateral millimetric pulmonary nodules largest seen at anterior segment of right lower lobe measuring mm compared to last study seen at left lower lung lobe superior segment measuring about 5 mm
- Still noted No pathological hilar or mediastinal lymph nodes (only tiny pretracheal L. N 5mm.).
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The visualized part of the skeleton shows:
  - Still noted left 7th rib posterior aspect well defined exophytic bubbly lesion showing internal calcification measuring about 3.4 x 1.6 cm causing compression and scalloping of the 6th rib likely of benign nature ? osteochondroma.
  - No suspicious osseous lesions.
  - Spondylodegeneartive changes.

Opinion:

- **Mild progression of bilateral millimetric pulmonary nodules .**
- **Stationary rest of the study.**

Dr. Reham Khedr,,MD

Signature Reeham.Khader
27/08/2025 9:32 AM

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL

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27/08/2025 9:32 AM

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Signature Reeham.Khader
27/08/2025 9:32 AM

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[[DOCTYPE:CT_SCAN]]

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Hospital:	Goustave Roussey International Egypt

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[[DOCTYPE:CT_SCAN]]

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Date:	2026-06-17
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

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Dr.sherif Wagdy

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Moneera Ahmed El-Sayed
Branch : Dokki

Date : 27-09-2025 10:49
Medical No. : 210289

NILE DIAGNOSTIC CENTER

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History

- A 49-year-old lady with history of left breast cancer, referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- Enhanced tracer uptake is seen at the posterior end of left 7th rib.
- No other abnormal active or cold focal bony lesions could be seen to account for metastatic osseous lesions.
- Degenerative changes are noted at the spine and big joints.

Impression:

- **Active osseous lesion at the posterior end of left 7th rib; ? likely metastatic, for correlation with radiological data.**
- **The rest of skeleton is free from metastases.**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient Name	: الأستاذه / منيره احمد السيد محمد احمد	Request No.	: 13000710
Patient No.	: 27311121303008	Request Date	: 15/10/2022
Path No.	: 3059/22	Verification Date	: 09/11/2022
Sex	: Female	Financial Category	: برام دار السلام - نقابة الدولة
Age	: 48 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed you

IMMUNOHISTOCHEMISTRY REPORT

CLINICAL DATA
History of left MRM , cervical LN enlargment

NATURE OF SPECIMEN
Cervical LN enlargment ; Excisional biopsy

GROSS EXAMINATION
* Fibrofatty tissue piece measured 3x3 cm collectively entangling 6 lymph nodes up to 0.5x0.5 cm .
* IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & KI67 clone (30-9).

MICROSCOPIC EXAMINATION
* Histological type: Metastatic carcinoma likely of mammary ductal origin in the received 6 lymph nodes (6/6)
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
* IPT (Positive control showed appropriate reactivity) :
* GATA3: Diffusely positve
* ER: Modrate nuclear reaction in 70% of tumor cells (Allred score 7/8).
* PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
* HER2: No membrane reaction (score 0)
* Ki 67: 10% nuclear reaction of tumor cells

DIAGNOSIS
Cervical LN enlargment ; Excisional biopsy:
Metastatic carcinoma , grade II of mammary ductal origin in the receiced 6 lymph nodes (6/6) .
IPT:
* GATA3: Positive

Verified by :
DR.Menna Nabil

Immunohistochemistry
[[DOCTYPE:IHC]]

Name : Moneera Ahmed El-Sayed
Branch : Dokki

Date : 27-09-2025 10:49
Medical No. : 211289

Tc^{99}m MDP SKELETAL SCINTIGRAPHY

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Dr. Alaa Nagi, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTRE

Call Center 19...

Bone Scan
[[DOCTYPE:BONE_SCAN]]